

FUTURE SCIENCE: Stem cell extraction and storage centre at AIIMS, Delhi



Rs 2 lakh 65 years

crore will be the estimated market size of healthcare by 2012

is the current life expectancy in India. It was 23 at the turn of 20th century.

hordes of medical tourists from around the world to India? A paradigm shift is at work. The healthcare market is on a boom—from Rs 92,700 crore in 2001 to an estimated Rs 2,08,800 crore by 2012. What brought it about? After all, government spending on healthcare has been historically low, averaging less than 1 per cent of the GDP. "It's the entry of big corporates in healthcare," say healthcare analysts Zubin Daruwalla and Sandeep Sinha of Frost & Sullivan (F&S). Many of these are top players in financial services, insurance or pharmaceu-

ticals. "They have the wherewithal to mobilise huge resources globally." The sheer deficit in patient-to-bed ratio has drawn global attention for investments to India. Now healthcare is being promoted as a branded product. And treatments are no longer focused on simple curing, but an array of services—from well-trained and cheerful paramedical staff to in-hospital ambience, facilities for patients' attendants and state-of-the-art equipment.

The first spurt was provided by Indian doctors returning home with techniques, skills and global expertise, to join the private sector. "That upgraded the standards in this country," says Dr Ashok Seth, chairman, Max Heart and Vascular Institute, Delhi. But technology formed the real backbone of the shift: evolution of life-support systems, early diagnosis mechanisms, expensive third and fourth generation medicines and greater number of devices-related surgeries. Cataract is a classic case. From a time-consuming surgery till the mid-'90s, it has now been reduced to a 15-minute procedure. Behind it all was the new-age consumer, obsessed with health and plagued by diseases triggered by new lifestyles. "People are more sensitive about matters concerning health and are more willing to spend on better facilities," point out Daruwalla and Sinha.

Healthcare as an industry is coming into its own. But what's the road ahead? Cardiac surgeon Dr Naresh Trehan is a believer in the levelling impact of market forces. "What we lacked was infrastructure. People were spending money for going to unsanitary places. Now that proper infrastructure is coming into place, we have to keep one eye on the costs and another on quality," he says. That includes more and better infrastructure, universal insurance to subsidise costs, indigenous manufacture of cutting-edge technology, legislations to standardise procedures and quality, corporates with sensible business models and ethical guidelines for doctors.

There's more. Trehan explains the blueprint of the "revolution", "The rich will patronise the private sector, the poor will flock the public sector, while subsidised hospitals under public-private partnerships will provide quality healthcare to the rest at reasonable rates." The new buzz in the air raises new hopes. Fingers crossed.

K. ANJI REDDY, Dr Reddy's Laboratory, 2005

"The future of medicine has two challenges: to harness science and to do it at an affordable cost."